

THE HORMONE RESET SERIES

Cycle Syncing

*How to Work With Your Hormonal Rhythm
Instead of Against It*

Disclaimer

This book is intended for educational and informational purposes only. The content does not constitute medical, clinical, or professional health advice. The author and publisher are not licensed healthcare providers.

Nothing in this book should replace consultation with a qualified physician, endocrinologist, gynecologist, or other healthcare professional. Hormone health is highly individual and requires personalized clinical evaluation.

Always seek the advice of your doctor or qualified health provider with questions you may have regarding a medical condition or before starting any new health regimen. Never disregard professional medical advice or delay seeking it because of something you have read in this book.

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The Infradian Rhythm You Didn't Know You Had

The 24-hour circadian clock gets all the attention. Women also operate on a 28-day ultradian cycle that affects everything from energy to cognition.

What the Monthly Cycle Actually Is

The menstrual cycle is not just a reproductive event. It's a monthly fluctuation in estrogen, progesterone, testosterone, LH, and FSH that affects brain function, metabolism, sleep, mood, physical strength, and immune function across its four phases.

Phase 1 (menstruation): low estrogen and progesterone. Energy typically lower, inward orientation natural. Phase 2 (follicular): rising estrogen, increased energy, improved cognitive sharpness and social ease. Phase 3 (ovulation): estrogen peaks, testosterone rises, typically the highest energy and most extroverted phase. Phase 4 (luteal): progesterone rises then falls, energy more variable, premenstrual symptoms most common in the late luteal phase.

These are not personality shifts. They are predictable neurochemical changes driven by hormone fluctuations.

"Your energy is not random. It's cyclical."

Exercise and the Cycle

Strength, endurance, recovery, and injury risk all fluctuate with the cycle. Training accordingly isn't extra — it's efficient.

Phase-Based Training

Follicular and ovulatory phases (days 1-14): rising estrogen supports muscle recovery, improves neuromuscular coordination, and elevates pain tolerance. This is the optimal phase for higher-intensity training, new PRs, and skill-intensive work.

Luteal phase (days 15-28): rising and falling progesterone increases core body temperature slightly, impairs heat dissipation, and elevates the stress response. Moderate-intensity training is often more productive than maximum effort. Research also shows increased ACL injury risk around ovulation — worth knowing for high-impact athletes.

There is no phase in which rest is required unless symptoms are genuinely limiting. The adaptation is about optimizing effort, not restricting it.

*"Syncing training to your cycle is not an
excuse to do less. It's a strategy to do more
where it counts."*

Nutrition Across the Phases

Metabolic rate, iron needs, and carbohydrate tolerance all shift across the menstrual cycle.

What Changes and When

Resting metabolic rate increases by approximately 100–300 calories in the luteal phase — meaning the body genuinely needs more fuel in the week before menstruation. Premenstrual increased appetite is a physiological response, not a lack of self-control.

Iron loss during menstruation warrants attention, particularly in women with heavy cycles. Iron-rich foods (red meat, shellfish, lentils, dark leafy greens) and pairing iron sources with vitamin C to enhance absorption support replenishment.

Magnesium needs may increase premenstrually — deficiency is associated with worse PMS symptoms including cramps and mood changes. Magnesium-rich foods include dark chocolate, pumpkin seeds, leafy greens, and almonds.

"Increased appetite before your period is your metabolism talking. Listen to it."

Mood, Cognition, and Hormonal Variability

Understanding why you feel different at different times of your cycle is powerfully normalizing.

The Hormone–Brain Connection

Estrogen increases serotonin receptor sensitivity and supports dopamine production — explaining improved mood and cognitive sharpness in the follicular phase. Progesterone has calming GABA-like effects but can also cause fatigue and, in women sensitive to progesterone fluctuations, premenstrual mood changes.

PMDD (premenstrual dysphoric disorder) represents the severe end of this spectrum — not 'normal PMS' but a clinical condition where the central nervous system responds abnormally to normal hormonal fluctuations. It affects 3–8% of women and is highly treatable. If premenstrual mood changes are significantly impairing function, clinical evaluation is warranted.

Tracking your cycle with symptom notes for 2–3 months reveals individual patterns that help distinguish cycle-driven variability from other mood conditions.

*"Knowing when you're in a vulnerable phase is
half the battle."*

Work and the Cycle

Applying cycle awareness to how you schedule work, social commitments, and rest is the practical heart of cycle syncing.

Scheduling With Biology

This is not about limiting yourself based on hormones — it's about scheduling higher-demand activities when energy and cognition are typically at their natural peak, and scheduling lower-demand or more creative work during phases when those capacities are naturally reduced.

Follicular phase: ideal for starting new projects, brainstorming, learning new skills, social commitments, difficult conversations. Luteal phase: ideal for detail work, finishing projects, solitary focus, administrative tasks. Menstrual phase: ideal for reflection, evaluation, planning, rest.

Most people find that they were already unconsciously adapting to some of this variability. Making it conscious allows you to design your schedule rather than react to your energy.

"Work with your cycle, not despite it."

Cycle Syncing in Practice

The first step is simply knowing where you are in your cycle. Everything else builds from there.

Starting Simply

Track your cycle for three months using a basic app or notebook: day 1 (first day of bleeding), approximate ovulation (usually days 12-16 for a 28-day cycle, confirmed by basal body temperature or cervical mucus changes), and subjective energy and mood notes.

After three months, patterns become visible. The tracking itself is revelatory for most people who have never thought about their experience as cyclical rather than random.

You don't need to restructure your entire life around your cycle to benefit from this awareness. Even small adjustments — scheduling the most demanding meeting in the follicular phase, planning rest on day one of your period, not making major decisions in the late luteal phase when anxiety tends to be higher — add up to a meaningfully more sustainable experience of life.

*"Awareness is the first adjustment. Everything
else follows."*
